

A • CONNECT TERTIARY HOSPITALS

ONE EXCHANGE. MORE ACCESS. ONE CONNECTED JOURNEY.

Why Nigeria's tertiary hospitals should connect

THE OPPORTUNITY

Each hospital remains independent—but patients, referrals, services and payments can move through a trusted Exchange.

- **More patients** — People can discover approved services, register, book and begin care without first travelling to the hospital.
- **More services** — Participating hospitals can make approved specialties visible and refer patients to one another when another centre is better placed to help.
- **One Exchange identity** — A patient carries one Smart Clinic identity that can be securely linked to the separate patient number held by each participating hospital.
- **One payment access layer** — Patients can view supported bills and pay at their convenience through a shared payment or wallet pathway that each participating hospital formally accepts and reconciles.

WHAT CONNECTION MEANS

A patient can move between participating hospitals without losing the story, purpose or next step of the care journey.

B • MAKE THE TERTIARY HOSPITAL THE STATE HUB

The hospital begins to serve the whole state.

The tertiary hospital becomes the leading referral, specialist and digital-access hub for its state—reaching far beyond those who physically enter its gates.

- **Connect the health system** — Secondary hospitals, primary centres, private hospitals, clinics, laboratories, pharmacies, professionals and health stations join approved pathways around the hub.
- **Connect communities** — Schools, hotels, employers, estates, churches, mosques, universities and associations become organised doors into prevention, checks, appointments and care.
- **Reach healthy people** — The hospital can support Stay Well and preventive programmes before illness becomes severe or expensive.
- **Coordinate care** — Find Care guides people to an appropriate service, while My Hospital connects patients to registration, appointments, bills, records and follow-up.
- **Lead beyond one state** — The hub can extend agreed specialist and referral corridors into neighbouring states while retaining its institutional identity and clinical authority.

THE NEW POSITION

Not simply a hospital inside the state—a hospital organising a trusted community of care across the state.

A legacy of access, growth and sustainable income

- **New reach and revenue** — Virtual access, diaspora pathways, referrals, appointments, approved services and digital collections can bring more patients and traceable institutional income.
- **Care without unnecessary travel** — Patients can connect virtually, prepare before attendance and return physically only when the pathway requires it.
- **An institution that outlives a tenure** — The Exchange, state network, patient access channels and reconciled revenue pathways continue after the CMD leaves office.

IMPLEMENTATION TARGET

A first working implementation can be targeted within 20 days, subject to prompt hospital approvals, stakeholder availability and technical readiness.

The CMD makes four decisions

1. Choose the date for the first virtual onboarding session.
2. Nominate the Head of Medical Records, Head of ICT and Head of Finance.
3. Attend two or three short ownership sessions covering the Exchange, records and technology/finance decisions.
4. Authorise the stakeholder schedule and virtual follow-up plan.

WHAT HAPPENS NEXT

Week 1: product learning and workflow decisions. Days 8–20: configuration, stakeholder sessions, testing, follow-up and controlled activation.

“LET US CONNECT OUR HOSPITAL, BUILD OUR STATE HUB AND CHOOSE OUR FIRST ONBOARDING DATE TODAY.”